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Name: سميه سيد نصر المايح National ID: 26601102101202	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



نوريس





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : سميرة سيد نصر العبدلي

Sex:

3- Medical number:

Female

83261739

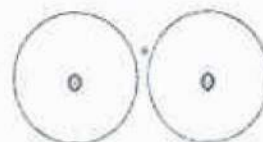
Old Med:

Old Med

544-2025

2-ID number: 26601102101202

Patient Age: 59 Y



Family history

Family history
-ve

Phone

Phone
01143116900

BMI

BMI

REASON OF MDT

REASON OF MDT
NOT CHECKED

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT
2025-05-22

Co morbidity

Co Morbidity
htn

Menopausal
status

Menopausal Status
NOT CHECKED

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

lt breast mass at 2 o'clock 2.8x3.7 cm BIRASD 4

Mets Workup :

cap: rt humer shaft extra osseous chondroid

BX IDC EX COMMENT	ER 8/8 EX COMMENT	PR 8/8 EX COMMENT	Her 2 Neu 1 EX COMMENT	SISH EX COMMENT	Ki67 25
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T 2	N 1	M 0
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Sonomammography

PATIENT HISTORY

lt wle+alnd: free surgical margin PT2(3.5CM), N1 LN 3/16 WITH EXTRACAPSULAR EXTENSION
PT2N1

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

ADJUVANT ACX4 TAXOTEREX4

PORT

ADJUVANT AI

ZOMETA/6MONTHS

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY Signature	RADIOLOGY	RADIOLOGY Signature
SURGERY	SURGERY Signature	PATHOLOGY	PATHOLOGY Signature
Psychiatric	Psychiatric Signature	Nutrition	Nutrition Signature

Name: سميه سيد نصر المليجي		
MR. : 931		
Detailed Medical Report		
		السيد
تحيه طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was diagnosed with Luminal B breast cancer, clinical stage cT2N0. An upfront wide local excision was performed in April 2025, revealing pathologic stage pT2N1 with three of sixteen nodes positive. The patient subsequently received adjuvant dose-dense doxorubicin/cyclophosphamide (AC) followed by paclitaxel (Taxoter), postoperative radiation therapy (PORT), and has been on an aromatase inhibitor (AI) since December 2025. Additionally, zoledronic acid (Zometa) was initiated in December 2025. In April 2026 imaging demonstrated a BI-RADS 1 assessment of the right breast and a BI-RADS 2 assessment of the left breast. The patient was prescribed aramidex with supportive care and instructed to continue this regimen for three months, with the next follow-up scheduled for 8 August 2026.		
تحريرا في ,		
	التاريخ :	20-09-2026
	الوقت :	
	حنان عزت شفيق تاودروس	الطبيب المعالج :
يعتمد ,		

Name

Date

16-1-2025

**DIGITALIZED LOW DOSE SOFT TISSUE MAMMOGRAPHY
WITH COMPLEMENTARY BREAST US EXAMINATION OF
BOTH BREASTS REVEALED:**

- ACRA.
- An well-defined hyperechoic lesion is seen at the UOQ of the left breast corresponding by US to well defined globular shaped heterogeneous lesion of mixed echogenicity partially solid partially cystic is seen at 2 o'clock with peripheral and internal vascularity and causing posterior shadowing. It measures 2.8x3.7cm.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complex.
- Bilateral non specific axillary lymph nodes showing preserved fatty hilum.

Opinion:

- Left breast UOQ suspicious lesion (BIRADS IV); for further assessment with Dynamic MRI and pathological correlation.

Much Oblige
Dr. Bony Sayed



S C A N

Name: Somaya Sayed Nasr

ID: 4337428

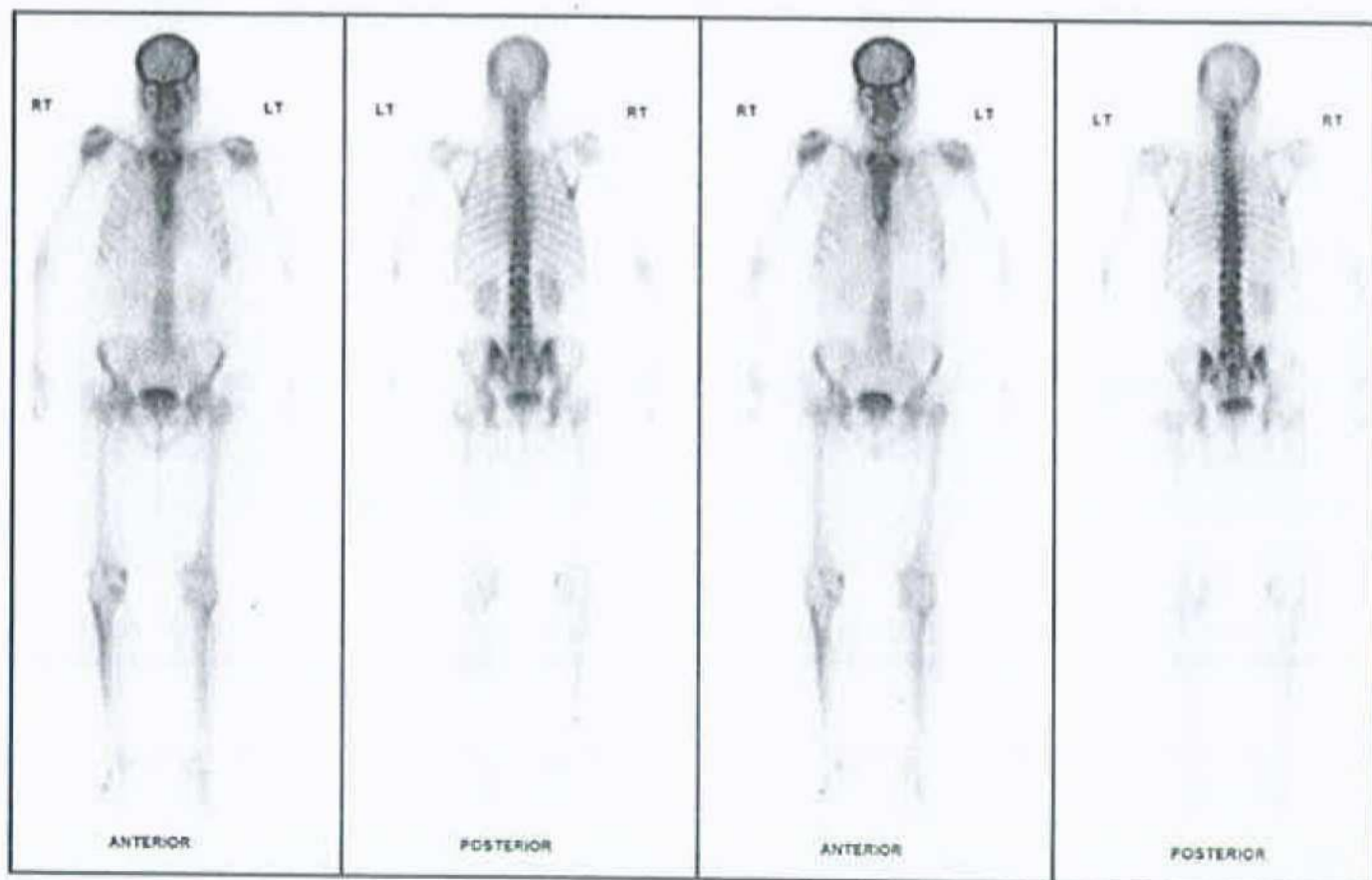
Date: 23/03/2025

 ^{99m}Tc -MDP W.B. Bone Scan**Clinical data:**

Indication	History of left breast cancer, with right humeral shaft lesion.
Tumor markers	Not available
Skeletal trauma/ surgeries	No
Previous imaging	CT CAP
Previous bone scan	No

Technique:

Injected dose	740 MBq
Images	Delayed whole body + spot images





S C A N

Findings:

- No abnormal tracer uptake is seen over the surveyed skeleton that would account for bone metastases.
- Normal visualization of both kidneys.

Impression:

- No scintigraphic evidence of bone metastases.

Dr. Sherif El-Refaei, MD, EBNM

Dr. Amany Ali, MSc



PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: سمية سيد نصر المليجي	Age: 59 years	Sex: Female
Request type: مبادره	Sub request type:	
Hospital no:	Pathology no.: 4-2025	
Department:	Date of biopsy: 20/1/2025	
Referred by: د. غاده خالد	Receiving date: 20/1/2025	
Nature of specimen: U.S Guided Biopsy.	I.D. Number: 26601102101202	

Clinical diagnosis: Left UOQ highly suspicious mass, (BIRADS4) for histopathological correlation.

Gross pathology :

Four gray white cores up to 1cm long each, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER- PR- Her2- Ki67

Microscopic picture:

Sections examined revealed breast core tissue showing infiltrating mammary carcinoma tumor forms sheets clusters, cords, single cells and cribriform pattern infiltrating among desmoplastic stroma. Tumor cells show moderate anaplasia with moderate tendency to form tubules and frequent mitosis. There is insitu componer intermediate grade solid pattern. No peritumor lymphovascular tumor emboli or perineural invasion could be seen in the core examined.

Immunohistochemistry: revised

ER. Strong positive nuclear staining in 90% of tumor cells (Allred Score 8/8).

PR. Strong positive nuclear staining of 70% of tumor cells (Allred Score 8/8).

Her2: Faint incomplete membranous staining in >10 of tumor cells (Score 1+) (Her2 low).

Ki67: Positive nuclear staining of 25% of tumor cells.

Pathology Lab. Manager

Nermine Nabil, M.D

Comments: Please correlate with the clinical, radiological, biochemical and serological findings.



PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

سميه سيد نصر 4

Diagnosis:

Left Breast Mass, Core Biopsy:
#Invasive Duct Carcinoma, grade II with insitu component.
(ER: 8/8),
(PR: 8/8),
(Her2: Score1+), (Her2 low)
(Ki67: 25%)

Signature:

Prof.Dr. Manal El Mahdy

Dr. Nermine Nabil , M.D.

Dr.Mona Mohamed. M.D

Pathology Lab. Manager

Nermine Nabil, M.D

Comments: Please correlate with the clinical, radiological, biochemical and serological findings.



Patient Name: Somaya Sayed Nasr El-Miligui	Date : 08/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 59 Year(s)
Medical ID: 544/2025	Pat. ID: 98259

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast UOQ irregular enhancing lesion measuring 2.5 x 2.1 cm.
- Multiple left axillary level I & II pathological lymph nodes largest measuring 1.5 x 1.1 cm.
- Bilateral scattered subpleural atelectatic bands.
- Bilateral few pulmonary nodules reaching 3 mm.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Right thyroid hypodense nodule measuring 1.5 x 1.6 cm.

II Abdomen & pelvis:

- The liver is enlarged with homogenous CT texture. No definite enhancing focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Evidence of hysterectomy with no sizable operative bed lesions.
- Normal appearance of the other pelvic organs.

Patient Name	: الأستاذة / سمية سيد نصر المليجي	Request No.	: 13000023
Patient No.	: 26601102101202	Request Date	: 27/04/2025
Path No.	: 1492/25	Verification Date	: 14/05/2025
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 59 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Up-Front.

Nature Specimen: Left breast, wide local excision, sentinel lymph nodes and axillary lymph nodes dissection Biopsies.

Gross Examination:

- * Left WLE specimen measured 9x6.5x3 cm, with covering skin ellipse measured 5.5x2 cm.
 - * Tumor extent: An ill defined firm grayish white mass.
 - Size: 3.5x3x2 cm.
 - * Skin involvement: Unremarkable grossly.
 - * Distance from margins: 0.2 cm from medial margin & 1 cm from skin.
 - * Left sentinel lymph nodes: A fibro fatty tissue piece measured 4x3x2 cm, entangling 5 nodules up to 2.4x1.5 cm.
 - * Axillary lymph nodes dissection: Multiple fibro fatty tissue pieces measured 6x6x1.5 cm, entangling 8 nodules up to 2.5x1.4 cm.
 - * Frozen sections results:
 - Free surgical resection margins.
 - Positive metastatic deposits in 3 out of 16 lymph nodes (3/16) with extracapsular extension.
 - * Designation:
 - 1-4: Mass.
 - 5: Lateral margin.
 - 6: Medial margin.
 - 7: Superior margin.
 - 8: Inferior margin.
 - 9: Deep margin.
 - 10: Sentinel lymph nodes.
 - 11-12: Axillary lymph nodes.
- Dissected by: Dr. Yasmine Essam.

Verified by :
Dr.Toka Medhat

Printing Date : 14/05/2025 11:23:59

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Printing By : marwa mohamed youss

Patient Name : الأستاذ / سعيدة نصر الميحي	Request No. : 13000023
Patient No. : 26601102101202 -	Request Date : 27/04/2025
Path No. : 1492/25	Verification Date : 14/05/2025
Sex : Female	Financial Category : نفقة الدولة - أورام دار السلام
Age : 59 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Microscopic Examination:

* Left breast, wide local excision, sentinel lymph nodes and axillary lymph nodes dissection

Biopsies:

* Histological Type: Invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.

* Ductal carcinoma in situ: Minimal (3%).

* Lymph node involvement: Positive metastatic deposits in 3 out of 16 lymph nodes (3/16) with extracapsular extension.

* Angioinvasion: Not detected.

* Perineural invasion: Not detected.

* Resection Margins: Free surgical resection margins.

* Microcalcifications: Not detected.

Diagnosis:

Left breast, wide local excision, sentinel lymph nodes and axillary lymph nodes dissection

Biopsies:

Invasive duct carcinoma, grade II, with minimal DCIS (3%).

- Free surgical resection margins.

- Lymph nodes: Positive metastatic deposits in 3 out of 16 lymph nodes (3/16) with extracapsular extension.

- Stage: pT2, pN1.

Signature

Dr. Marwa Mahmoud

Dr. Yasmine Essam

Verified by :

Dr.Toka Medhat

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